

D9 — GI Consult

Dr. H. Nakamura, Gastroenterology | Gastroenterology | 02/21/2026 06:30

Patient: Pemberton, Harold J. **MRN:** 847-29-3156 **DOB:** 04/12/1951 **Allergies:** Sulfa drugs (rash)

GASTROENTEROLOGY CONSULT NOTE

Patient: Harold J. Pemberton | MRN: 847-29-3156
Date: 02/21/2026 06:30 | Provider: Dr. H. Nakamura, Gastroenterology
Reason for Consult: URGENT – Upper GI hemorrhage

CLINICAL CONTEXT:
Nursing reported melanotic stool overnight. Patient had one episode of coffee-ground emesis this AM. Hemodynamically: HR 108, BP 110/68 (baseline 140s). AM labs show Hgb drop from 11.2 (admission) to 7.4. Cr 1.8 (rising from baseline 1.5).

EGD FINDINGS (performed 02/21/2026 08:00):

- Esophagus: Normal. No varices.
- Stomach: Single gastric ulcer located on the lesser curvature of the gastric body, approximately 3 cm in diameter. Forrest Classification IIa – visible vessel, not actively bleeding at time of endoscopy. Two endoclips placed successfully with primary hemostasis achieved.
- Duodenum: Normal.
- Biopsies: Taken from ulcer margins for H. pylori testing. Results pending.

ETIOLOGY:
Likely multifactorial:

- Therapeutic anticoagulation (apixaban)
- Systemic corticosteroids (methylprednisolone → prednisone)
- Possible NSAID use (ibuprofen on home medication list – unclear if taken recently prior to admission)

RECOMMENDATIONS:

1. IV pantoprazole 80 mg bolus, then 8 mg/hr continuous infusion
2. NPO until hemodynamically stable x 24 hours
3. HOLD anticoagulation – recommend minimum 72 hours post-hemostasis before considering restart. Discuss timing with cardiology given afib and stroke risk.
4. Strongly recommend against restarting any NSAIDs. Patient has multiple risk factors for recurrent GI bleeding (anticoagulation, steroids, prior ulcer now documented, CKD).
5. Continue prednisone – cannot abruptly discontinue given adrenal suppression risk after 3 days IV steroids. PPI coverage is essential.
6. Type and screen, transfuse for Hgb < 7 or active hemodynamic instability.

7. Serial CBC Q6H x 24 hours.

Will follow daily. Repeat EGD if evidence of re-bleeding.

Electronically signed by: Dr. H. Nakamura, MD

Date: 02/21/2026 09:45

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